



Psychiatry Workbook



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INSTRUCTIONS:

- Our initiative is to make a workbook in a precise manner to make you write the most important points, focus more on understanding the important concepts in class, save you precious time & energy, at the sametime keep you actively involved in the sessions.
- Workbook spaces are provided wherever necessary to help place your desired content.

PLEASE NOTE:

- This workbook has to be strictly used when the faculty is teaching as there are blank spaces which need to be filled, otherwise the inference may be misleading.
- The information contained in this book is strictly for educational purposes. Content is not intended as a substitute for professional medical advice, diagnosis or treatment.
- Every effort has been made to ensure the accuracy of information in this book. Neither the author nor the publisher are responsible for any errors or omissions. Any liability resulting from applying the ideas or information from this book, are strictly the responsibility of the reader.

DISCLAIMER:

- The notes have been made taking care of all the relevant points required but special care has been taken to avoid exhaustive nature. These notes are only for the purpose of understanding in a crisp manner and help you in quick complete revision. Please do not use them for the treatment of your patients as each patient requirement differ at various times.



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INTRODUCTION TO PSYCHIATRY

- Is a medical speciality concerned with
.....
.....
- Father of Psychiatry -
- 1856-1939



◀ FIG 1.1 Dr Sigmund Freud

- Father of Indian Psychiatry -

- 1st Psychiatrist -

- Father of Modern Psychiatry -

- Term Psychiatry coined by -

- Psych -

- Iatry-

- BRAIN =

= MIND has -

3 components

☐ Cognition =

☐ =

☐ = →

..... Psychiatric disorder/ Mental disorder

HISTORY IN PSYCHIATRY

- Name / Age/ Sex/ Education

-

-

-

-

-



.....

.....has No role in Psychiatry.

ONSET

It's the duration of

- Abrupt -
- Acute -
- Insidious or chronic -

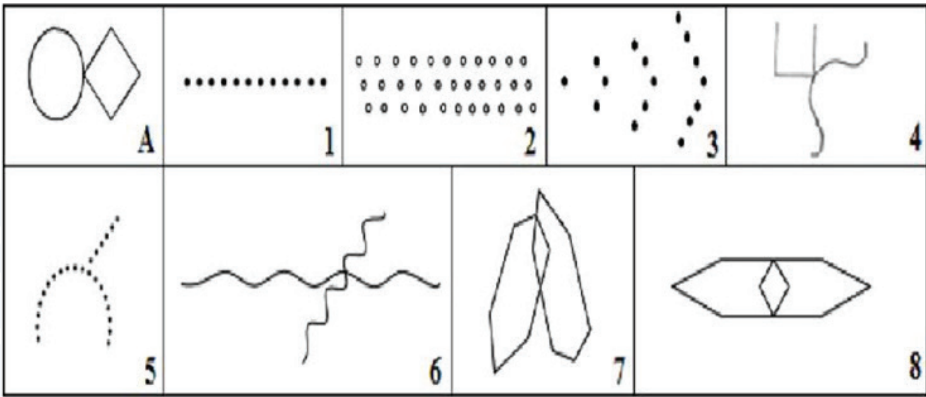
Prevalence of any mental disorder in India in Age > 18yrs → Excluding Tobacco →%

NMHS - NIMHANS (National Mental Health survey-2016)

- All mental disorders were classified in 2- major groups;

ORGANIC MENTAL DISORDER	FUNCTIONAL MENTAL DISORDERS
•	
.....	
•	
.....	
•	
.....	
•	
.....	
•	
Eg;	
•	•
•	•
•	•
.....	

-is the most common Psychiatric disorder seen in Medical & Surgical ward.
- Most common Test used to Screen organic Mental disorder is →



◀ FIG 1.2 BG Test

Ask the pt to draw these figures.

TYPES OF FUNCTIONAL MENTAL DISORDER

On the basis of presence or absent of.....

PSYCHOSIS	NEUROSIS
..... ●	
..... ●	
..... ●	
..... ●	

❑ CLASSIFICATION SYSTEM IN PSYCHIATRY

ICD-11	DSM-5	RDoC
❑	 in 2013	❑ ❑ only for ❑ NIMH (USA)

MENTAL STATUS EXAMINATION

In medicine we do physical examination similarly in psychiatry we do MSE---

1.

2. Psychomotor activity



3. ETEC----

❑

4. Speech

❑

❑

❑

❑

..... ↑

↓

5. Mood →

↓

• Happy // Sad/ depressed/

• - Normal

Active space

[illegible]

➤Affect.....

➤Affect.....

a.(flow) of thought

- b. Possession of thought
- c. Content of thought
- d. Form of thought

☐ Stream of thought☐ Thought

➤

.....

.....

.....

.....

.....

➤

➤

.....

➤ Circumstantiality---

.....

☐ Continuity of thought

➤ 1.

.....

.....

➤ 2.....

.....

Disorder of possession of thought

- Sometimes person loses control on their thinking and they have

☐

Seen in

Or

☐ Thought Alienation

.....

☐ DISORDER OF CONTENT OF THOUGHT

DELUSION

It.....

☐ DISORDER OF FORM OF THOUGHT

1.

2.

3.
4.

8. "PERCEPTION";

□
.....

□ ILLUSION

□

□ HALLUCINATION

□

9. COGNITION Assessment of Concentration/ Immediate memory

.....

Immediate memory.....

10. ABSTRACT REASONING/ ABSTRACTION

Similarity testing b/w objects

Proverb interpretation

BARKING DOGS SELDOM BITE

3 similarity b/w

Bus -

..... - orange

Bird- Aeroplane

11. INSIGHT

Awareness & understanding about the illness which Pt has

Grade I - complete denial of illness(absent insight)

Grade II -

Grade III -

.....

Grade IV-

.....

.....

.....

Grade V - True Emotional insight

12. JUDGEMENT

SOCIAL JUDGEMENT.

TEST JUDGEMENT

Workbook Questions

Q.1 A patient of schizophrenia says, "Lord hanuman was a celibate, I am celibate too, therefore I am lord hanuman". This is an example of:-

- (a). Loss of association
- (b). Autistic thinking
- (c). Neologism
- (d). Verbigeration

Q.2 Illusion is a disorder of?

- [A] Insight
- [B] Thought
- [C] Affect
- [D] Perception

Q.3 A young man presents to the psychiatry OPD reporting that whenever he sits down, he hears birds flapping their wings he believes this is a sign from God directing him to move in a specific direction to stay safe. He also describes that when he goes out, birds chirping is another way God communicates with him. Which of the following psychopathological phenomena does this scenario best represent?

- [A] Sudden delusional idea
- [B] Delusional perception
- [C] Paranoid delusion
- [D] Thought insertion

Q.4 Hypomimia is a disorder of?

- [A] Cognition
- [B] Affect
- [C] Speech
- [D] All of the above

Q.5 Which is not a disorder of 'Tempo' of thought is

- [A] Circumstantiality
- [B] Tangentiality
- [C] Prolixity
- [D] Retardation of thinking

Q.6 Auditory hallucinations in a patient suggest?

- [A] Functional cause
- [B] Neurosis
- [C] Organic cause
- [D] None of the above



Schizo -

Phrenia -

➤thought disorder.

Who had coined the term Schizophrenia



◀ FIG. 2.2 EUGEN BLEULER

Active space

4A'S OF BLEULER

.....

.....

.....

.....



Note: Not one of the 4 A's

.....

AUTISM

Because of defective way of thinking pt develops

Poor interaction

Poor contact

↓
ed Verbal output

Remains alone

AMBIVALENCE

- ☐ State of confusion in mind
- ☐ Inability to decide for or against particular task or activity

SECONDARY SYMPTOMS OF SCHIZOPHRENIA

☐

☐

KURT SCHNEIDER

".....

First Rank symptoms of schizophrenia" (SFRS)

1. HALLUCINATION**AUDITORY HALLUCINATION**

1st person →

(Hearing of one's own thought loudly)

2nd person →

.....

3rd person →

2. THOUGHT PHENOMENON

☐ Thought

☐ Thought

☐ Thought

3. MADE PHENOMENON

☐ Made

☐ Made

☐ Made

4. DELUSIONAL PERCEPTION

.....

.....

Active space

5. SOMATIC PASSIVITY

Total - 11 symptoms

3 hallucinations

3 thought phenomenon

3 made phenomenon

Delusional perception

Somatic passivity

EPIDEMIOLOGY

- ❑ Prevalence of schizophrenia

In General population:.....100)

- ❑ Prevalence of schizophrenia & other psychosis in India

According to NMHS is.....

Incidence to schizophrenia is

- ❑ M.c age of onset :

- ❑ Male= Female

ETIOLOGY**1. GENETIC**☐ Polygenic inheritance

- ☐ Alpha 7 Nicotine Receptor Gene
- ☐ C-OMT Gene
- ☐ Neuregulin -1Symptoms
- ☐ Dystrobrevin -1 Symptoms
- ☐ DISC - I

(Disrupted in Schizophrenia)

Chromosome.....&..... is associated with Schizophrenia

If one parent is suffering from Schizophrenia risk in their children

If both parents

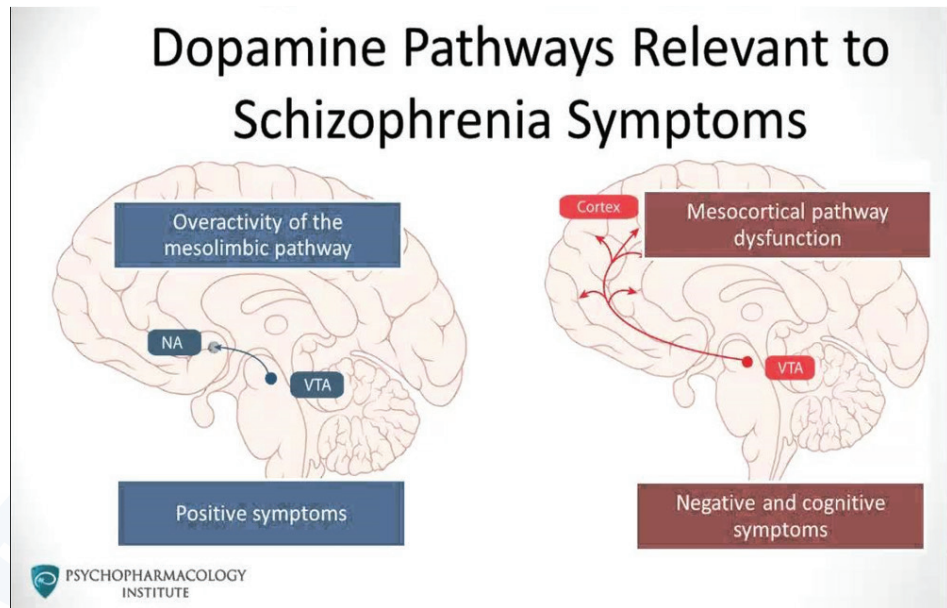
In Monozygotic twins of affected parent

If one sibling is having schizophrenia risk in another sibling is

2. BIOCHEMICAL

- ☐ Increased
- ☐ Increased
- ☐ Decreased

Active space



MESO LIMBIC PATHWAY

MESO CORTICAL PATHWAY

From Mid Brain to cortex

**NEGATIVE SYMPTOMS**

- Anhedonia---
- Avolition / Apathy
- Poverty of
- Asociality →
- Attention Deficit →

Active space

MIST

- Ambivalence →
- Affective flattening →

These symptoms to

Treatment (1st generation anti-psychotic drugs)

1st Generation anti-psychotic drugs A/K as Neuroleptic drugs

3. ENVIRONMENTAL FACTORS

- ❑ Schizophrenia is more common in person having ".....
Birth"
Because of Risk of infection to mother is
highest during
- ❑ History of obstetric complication during Birth.
- ❑ Left Handedness, Advanced.....age
- ❑to another place
- ❑personality
↓
.....
- ❑ Drugs →
Produce Paranoid Schizophrenia like symptoms

4. PSYCHODYNAMIC THEORY

According to Dr.Freud

Schizophrenia develops because of Regression into.....
stage of Psycho sexual development

5. FAMILY THEORY

- ♦ Expressed Emotions
 - ❑ When parents / Care giver behave with the patient.
..... of Schizophrenia

6. PSYCHO NEURO IMMUNOLOGY

- ❑ Decreased production of
- ❑ Decreased activity of due to increased activity.

❑ ONSET OF SCHIZOPHRENIA

Childhood Onset.....

Early onset →

Late onset →

SUICIDE IN SCHIZOPHRENIA

- Single most common cause of pre mature death in pt's of Schizophrenia is.....
- Risk of suicide
- Actual Rate of suicide

ANTI SUICIDAL DRUGS

-
-
-

DIAGNOSTIC CRITERIA

◆ Symptoms

- a.
- b.
- c.
- d.
- e.

At least 2 symptoms out of which at least 1 symptom must be from 1st three symptoms

♦ ICD-10/ICD-11

If symptoms are present for

.....→ Schizophrenia

.....→ Acute & Transient Psychotic Disorder

DSM - 5

Duration of Symptoms

..... Schizophrenia

.....Schizophreniform Disorder

..... Brief Psychotic Disorder

Symptoms of Schizophrenia

HALLUCINATION

❑ It isperception

(Note: This statement is)

CHARACTERISTICS OF HALLUCINATION

1. As real/vivid as

2. Occurs inmind

3. Not underof person

4.are not involved.

5. Insight is

6. Occur in

7.

When Hallucination occurs in

..... present..... Hallucination

Not under

TYPES OF HALLUCINATION

- Auditory
- Visual
- Gustatory
- Tactile
- Olfactory

SPECIAL TYPES OF HALLUCINATION

Functional

- Require stimuli to occur

-
.....
.....

Reflex / Synesthesia

- Require stimuli

-
.....
.....

Extracampine type

- When source of Hallucination is beyond sensory field or beyond normal Hearing range.

Cenesthetic

- Type of Hallucination
- sensation in Brain/ Head
- Cutting sensation of
- Pushing sensation in abdomen

DELUSION

False, firm, fixed,
.....
.....

TYPES OF DELUSION

1. DELUSION OF

-
- It is 2 types-
D.....
D.....

2. DELUSION OF

-

3. DELUSION OF

-
-
-

4.SYNDROME

OR

MORBID JEALOUSY

OR

CONJUGAL PARANOIA

OR

DELUSION OF

-
-

5. COTARD SYNDROME

OR

DELUSION OF

- Seen in
- Pt. feel that

6.SYNDROME OR DELUSION OF DOUBLE OR
..... SYNDROME

-person has been replaced by some.....person.

7.SYNDROME

-person has been replaced by some.....person.

8. Delusion of grandiosity

-

9. SYNDROME

OR

DELUSIONAL

- SIGN

DISORGANIZED SPEECH/ THOUGHT

- Derailment
- Loosening of association
- Tangentially
- Circumstantiality
- Flight of ideas
- Perseveration

DISORGANIZED BEHAVIOR

- Muttering to self / talking to self
- Laughing to self
- Not
- Not take bath

Active space

NEGATIVE SYMPTOMS

SUB - TYPES OF SCHIZOPHRENIA

According to ICD-11/ DSM- 5

- No subtypes of Schizophrenia

❑ ICD - 10- SUB TYPES OF SCHIZOPHRENIA

Based on Clinical presentation

♦ Paranoid Type

❑

.....

Characterized by

- ❑ Late onset
- ❑ More positive symptoms
- ❑ prognosis

♦ DISORGANIZED TYPE

(..... TYPE)

Coined by EWALD HECKER

- 2nd Most common

—

- Disorganized thoughts, Behaviour

—

—

—

It has **WORST PROGNOSIS**

♦ **CATATONIC TYPE / CATATONIA**

♦ (In ICD 11- CATATONIA)

.....

Cata - Disturbed

Tonic - tone

— Characterized by Marked disturbance in

Or

— Defect in conation

— Term catatonia coined by → **KARL KAHLBAUM**

2 types of catatonia-

EXCITATORY	STUPOROUS
— Increased..... — This activity is Non - Goal directed & has no apparent relationship with external environment.	Stupor ↓ ↓ Akinetic + Mutism (Absence Of..... ) — Echopraxia — Echolalia — Posturing — Catalepsy — Waxy flexibility — Mitgehen — Negativism — Rigidity — Gegenhalten — Automatic obedience — Ambitendency — Psychological pillow

MIST

👉 TREATMENT

— DOC → inj.

After 15-30mins



Improvement

No improvement



positive

..... is treatment

challenge test

Catatonia has.....PROGNOSIS

Simple Subtype ----

There are 2 more sub types of Schizophrenia as per Tim Crow---

❑ Treatment of Schizophrenia-----

❑ DOC -----

- 1st episode - Treatment should be continued for at leastyears
- Multiple episode -

❑ Antipsychotics-----

1 st Generation	2 nd Generation
Or	Or
.....Anti - Psychotics	Atypical Anti- Psychotics
Or	Or
..... Antagonist (DRA)	Serotonin Dopamine Antagonist (SDA)
Or	
Neuroleptics	
D2 Inhibitors	
❑ Efficacy starts when.....of Dopamine receptors are blocked	❑ Block 40% Receptor
❑ Side effects starts when more than > D2 receptors are blocked.	+
	 receptor Antagonist
	❑ Less side effects

❑ All anti Psychotics belongs to Pregnancy -

Except CLOZAPINE & LURASIDONE

PREGNANCY -

1st Generation anti Psychotics are ----

2nd generation anti psychotics are ----

SIDE EFFECTS OF ANTI PSYCHOTIC DRUGS

1. Acute dystonia -

☐ contraction of

☐

Caused due to D2 # in Pathway

(Extra Pyramidal system)

Centrally acting drugs

☐

☐ Procyclidine

☐

To reverse acute dystonia immediately

☐ Inj.....

2. AKATHISIA

☐ of anti - Psychotics

Symptoms

☐ Subjective feeling of or

Objective signs of restlessness pacing / tapping feet

DOC -

3. TARDIVE DYSKINESIA

OR

RABBIT SYNDROME

OR

THORAZINE SHUFFLE

☐ Late side effect of anti-Psychotics

☐

.....

Most common is in the form of

☐

☐

☐

Active space

Tardive dyskinesia develops after 3 months to 1 year of
anti - psychotic use.

👉 ETIOLOGY

- ❑ Development of super sensitivity by post - synaptic
..... Receptors

👉 TREATMENT

- ❑ Shift from 1st Generation to 2nd Generation
- ❑ DOC - FDA approved ----.....

👉 Other drugs

- ❑ Valproate
- ❑ Tetrabenazine

4. Neuroleptic Syndrome

5. Metabolic syndrome -

❑ CLOZAPINE

Belongs to pregnancy-B

Indications for Clozapine --

1. It is the forschizophrenia
2. Effective in
3. Least risk of
4. It is F. D A approved agent

Active space

☞ Side effects:

1.
 2.
 3. Metabolic syndrome
 4. (neutropenia and thrombocytopenia) is an
..... and it is an independent side effect of clozapine.
- ☐ Myocarditis
 - ☐ Tachycardia
 - ☐ Increased temperature
 - ☐ T wave inversion is seen in E. C. G

☞ SEIZURES

- These are side effects of clozapine. It is seen at dose above

Good and bad prognostic indicators of schizophrenia

☐ Good prognostic indicators

- ☐
- ☐
- ☐
- ☐
- ☐
- ☐

☐ Bad prognostic indicators:

- ☐
- ☐

- ❑ _____
- ❑ _____
- ❑ _____
- ❑ _____
- ❑ _____

PERSISTENT DELUSIONAL DISORDER

- Diagnostic criteria
- 1. Presence of for at least 3 months according to ICD-11
- Or for at least one month according to DSM-5
- Differentiation of delusional disorder from schizophrenia
- Absence of hallucination
- No negative symptoms like emotion, personality, work, sleep
- Daily routine activities (normal)
- Delusion of is the most common type of delusional disorder

Treatment of Delusional Disorder—

Workbook Questions

Q.1 Most Common side effect of Clozapine is?

- [A] Seizures [B] Agranulocytosis
- [C] Sialorrhea [D] Metabolic syndrome

Q.2. A patient on haloperidol, presents to you with fever, rigidity and raised CPK, autonomic instability. Treatment ? (Psycho-pharmacology)

- [A] Stop antipsychotic and start dantrolene/bromocriptine
- [B] Continue antipsychotic and conservative treatment with antibiotics, hydration
- [C] Benzodiazepines
- [D] Anti-cholinergics

Q.3 A 26 year old male believes that he is very rich, think people jealous of him and that someone is trying to kill him and take his wealth. He appears depressed and physically unwell. Family is concerned about his well being. What is the most likely clinical presentation?

- [A] Delusion of grandiosity with persecutory delusions
- [B] Delusion of grandiosity with nihilistic delusions
- [C] Persecutory delusions with depression
- [D] Delusion of grandiosity with reference

Q.4 A 29-year-old man with schizophrenia, who has been stable on risperidone for several months, recently developed dystonia. To manage this, his psychiatrist prescribed benztropine 2 mg twice daily. One week later, he presents to the emergency department complaining of dry mouth, blurred vision, confusion, and urinary retention. On examination, he is disoriented, febrile, and tachycardic, with dilated pupils and absent

bowel sounds. What is the most appropriate next step in managing this patient?

- [A] Discontinue risperidone and initiate lorazepam for possible neuroleptic malignant syndrome (NMS)
- [B] Administer IV dantrolene and initiate cooling measures for suspected serotonin syndrome
- [C] Start physostigmine with continuous cardiac monitoring for anticholinergic toxicity due to benztropine overdose
- [D] Administer activated charcoal and IV fluids; observe the patient as symptoms are likely self-limiting

Q.5 Which anti psychotic drug causes least weight gain?

- [A] Aripiprazole
- [B] Cariprazine
- [C] Clozapine
- [D] Ziprosidone



MOOD DISORDERS

There are types of mood / Affective disorders

Depression

It is known. As major depressive disorder in

EPIDEMIOLOGY

Lifetime prevalence of depression in general population is..... which is highest among all psychiatric disorder

Males : Females ::

☐ Etiology

Diagnostic criteria for depression:

There are major and minor symptoms

☐ There are 3 major symptoms:

-
- Anhedonia
-

☐ There are 7 minor symptoms

- ❖ Either
- ❖ Either sleep
- ❖ Psycho-motor
- ❖ Decreased ability
- ❖ Feeling of
- ❖ Recurrent
- ❖ Low

Active space

Cognitiveor triad was given by

.....According.....to.....
 patients with depression develops defective way of

Also k/as

They develop negative view about-known as

They develop negative view about known as

And Negative view about known as.....

☐ Suicide in depression

..... is the most common cause of suicide

☐ Risk factors for suicide --

☐ Types of suicide-

According to NCRB.....

Current suicide rate in India is/lack population

Most common method is used

BIPOLAR DISORDER

'Mania'

It is a Bipolar disorder

□ Epidemiology:

- 0.8-1%
- It is in males and females
- Mania is more common in person with high socioeconomic status

ETIOLOGY-

☞ Symptoms:

- Major symptom:
..... in most of the day or increased

☞ Minor symptoms:

D

I

G

F

A

S

T

MIST

Mania is diagnosed if:-

- 1 major plusminor symptoms are required if we are considering mood as an Elevated Mood
- Or
- 1 major +minor symptoms are required if we are
- taking mood as irritable mood
- 7 days or symptoms of any duration if patient requires hospitalization
- If the same symptoms of mania are present with lesser Severity for days then it is hypomania

Recurrent depressive disorder ---

☐ 'Rapid cycling Bipolar disorder'

When there is episode of either
 inmonths period
 Common in

Treatment of Depression:

Drug of choice is

In Mild Depressive Episode ----specially during pregnancy and breast feeding -----treatment of choice is

☞ Tricyclic antidepressant

- ◆ Mechanism of action--

☞ Selective serotonin reuptake inhibitors

☞ Side effects:

- ◆ 'Serotonin norepinephrine reuptake inhibitor'

- ◆ Venlafaxine
- ◆ Desvenlafaxine
- ◆ Duloxetine
- ◆ Milnacipran
- ◆ Levomilnacipran

☞ Side effects

- Hypertension
- ◆ Norepinephrine and dopamine reuptake
 - Drugs is.....
 -

☞ Side effects

Seizures - so contraindicated in patients of epilepsy

- ◆ Norepinephrine reuptake
 - Drugs:
 -
- ◆ Noradrenaline + specific serotonergic antidepressant
- ◆ All antidepressant drugs

.....

.....

ECT-Electroconvulsive therapy in depression is

- Treatment of choice for depression
- Depression with
- Stuporous depression
- Depression with psychotic symptoms

♦ Treatment of Bipolar disorder:

.....of relative as well as patient about the illness is very important for good compliance of treatment

Drug of choice for Acute episode of mania is second generation Antipsychotics preferably

.....

Drug of choice for Acute episode of mania in pregnancy is

.....

.....

.....

.....

♦ Lithium

It is available as a tablet of Lithium carbonate

♦ Mechanism of action: Exact mechanism is unknown

♦ Indications of lithium -

1.

2.

3.

Side Effects of Lithium

L
I
T
H
I
U
M

Lithium has narrow therapeutic index -

Serum lithium levels are checked

..... days after starting Li+ and Blood sample is withdrawn

..... hours.....

Sign/Symptoms of Li+ Toxicity

Active space

♦ Sodium valproate

Indication:

1. Acute episode of mania when mood is irritable or dysphoric

2.

3.

S/E

Psychiatric disorder in Post Partum period -----

PERSISTENT mood disorder

1. Dysthymia.....

2. Cyclothymia:

It is characterized by numerousepisodes
of..... and mild

Adjustment Disorder-

Active space

Workbook Questions

Q.1 A 40-year-old woman with bipolar disorder is planning a pregnancy. Which of the following drugs must be avoided due to the possibility of Neural Tube Defects?

- [A] Valproate
- [B] Oxcarbamazepine
- [C] Lamotrigine
- [D] Levetiracetam

Q2 Which new psychoactive substance is found to have a rapid acting antidepressant action?

- [a] Ketamine
- [b] Cannabinoids
- [c] Bupropion
- [d] Mephedrone

Q3 A 28 year-old Office worker comes to the psychiatrist clinic with complaints of not being able to work properly and difficulty balancing family and work performance, stress from past 3 months. He had undergone financial crisis too. What is the most likely disorder the patient is suffering from?

- [A] Acute stress syndrome
- [B] PTSD
- [C] Adjustment disorder
- [D] General Anxiety disorder

Q 4 A 36-year-old woman presents with hyperactivity, No need for sleep, and excessive talking spending over the past 2 weeks. When questioned, she becomes irritable and angry. What is the most likely diagnosis?

- [A] Dysthymia
- [B] Bipolar I disorder with manic episode
- [C] Persecutory delusions with depression
- [D] Delusion of grandiosity with reference.

Q5: Which of the following is not included in ICD-11 core criteria of depression diagnosis?

- [A] Low mood for most of time of day
- [B] Loss of interest or pleasure
- [C] Loss of self esteem or confidence
- [D] Decreased energy levels

Q6 A 28 year old mother was diagnosed with mild depression, she has a 3 month old child. Which among the following therapy should be prescribed ?

- [A] Cognitive behavioural therapy (CBT)
- [B] Cognitive behavioural therapy + anti depressant
- [C] Antidepressant alone
- [D] Electroconvulsive therapy



- ❑ In treatment of mental disorders

.....

.....

ELECTRO CONVULSIVE THERAPY (ECT)

- Discovered by in 1938
- Mechanism of action-

INDICATIONS OF ECT

- 1..... M.C indication for ECT
- a.idea
- b. Severe.....symptoms

c. Stuporous depression

d.

2. Schizophrenia

3. Catatonia

4. Mania

5. Treatment

6. Delirium

7. Neuroleptic malignant syndrome

ECT is Treatment of choice in.....pregnant women or in pregnancy especially in

❑ ECT is the procedure during pregnancy

ECT is not indicated /not effective in;

A-

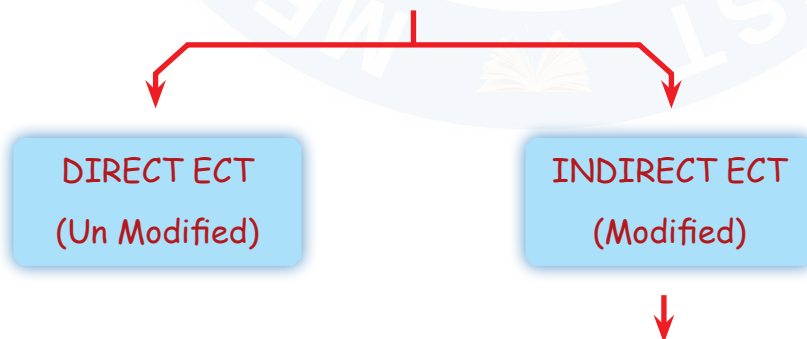
S-

P-

D-

.....

TYPES OF ECT



All ECT used now are Indirect ECT / Modified

General anaesthesia + Muscle Relaxant

MIST



1. Methohexitone.

Or

Methohexital

(Most common GA)

2. Thiopentone

Direct ECT → direct electric current

➤ Now a days Direct ECT is contraindicated



Succinylcholine

♦ CONTRAINDICATION FOR ECT

ABSOLUTE C/I



NONE

Previously ↑ was the absolute C/I

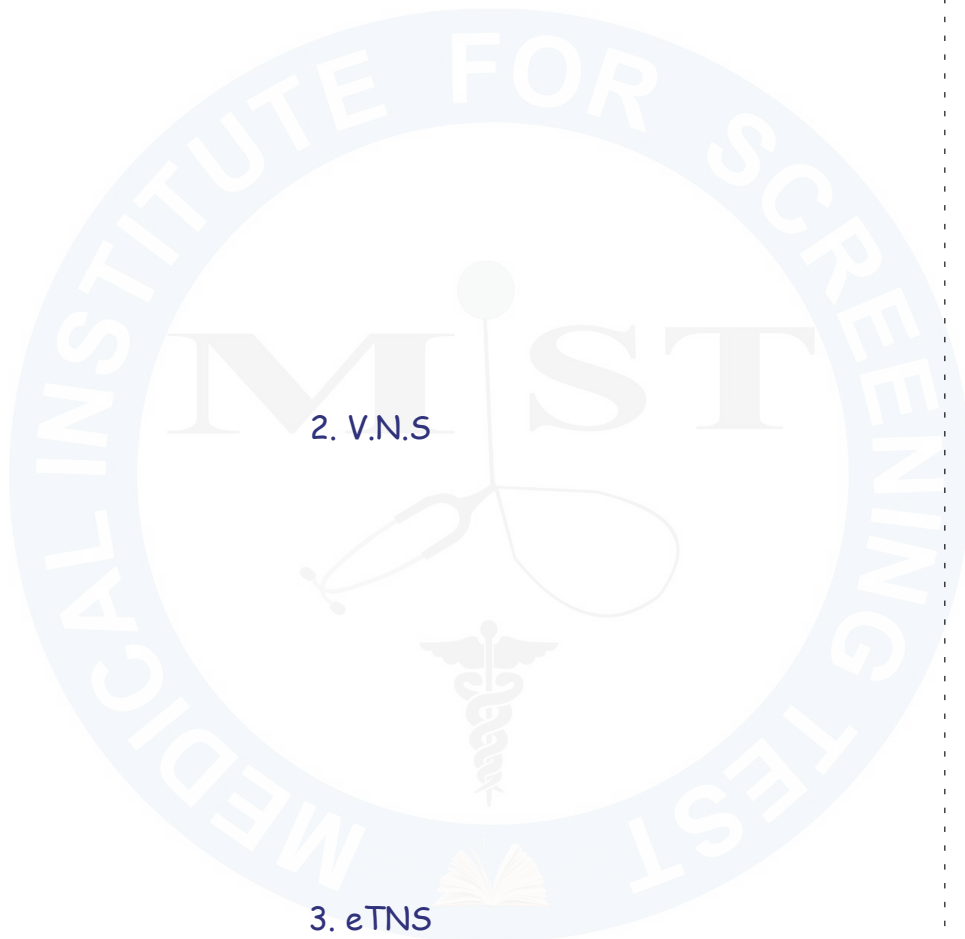
RELATIVE C/I	
1.	↑
2.	
3.	
4.	
5.	
6.	
7.	
	
	

Side effects of ECT

1. rTMS

2. V.N.S

3. eTNS



Workbook Questions

Q.1 Most Common indication for ECT is-

- [A] Hysteria
- [B] Depression with suicidal tendency
- [C] Chronic schizophrenia
- [D] Mania

Q.2 Which of the following increases on Electro convulsive therapy-

- [E] 5- Hydroxy- indole- acetic acid
- [F] Dopamine
- [G] Serotonin
- [H] Brain derived Neurotrophic factor

Q.3 Which biological therapy is used for resistant cases of ADHD-

- [A] VNS
- [B] eTNS
- [C] rTMS
- [D] ECT

Q.4 Repeated Transcranial magnetic stimulation (r-TMS) of Brain is used in the treatment of-

- [A] Depression
- [B] Resistant Schizophrenia
- [C] Obsessive compulsive therapy
- [D] Acute psychosis

Q.5 Identify the therapy?

- [A] ECT
- [B] eTNS
- [C] VNS
- [D] rTMS



'NEURO COGNITIVE DISORDER' OR ORGANIC MENTAL DISORDER

Cognition include

.....

DELIRIUM

- life threatening but reversible disorder of central nervous system
- It is characterized by in the level of and with impairment in

Features: clouding of consciousness

Delirium produces abnormal activity

disturbances + cycle impairment

Delirium is most common organic mental disorder.

👉 Etiology:

1. Main cause of delirium is

Dyselectrolytemia

Congestive heart failure

After surgical procedure specially after cardiac surgery

Either intoxication or withdrawal from the effect of pharmacological agent or drug.....

2. Main Neurotransmitter responsible for delirium is decreased

.....

👉 Investigation/Diagnosis--

👉 Treatment:

1. Treatment of underlying cause
 2. Anticholinergic cause: physostigmine salicylate (acetylcholine sterase inhibitor)
 3. For psychosis and agitation that develops after Surgery/Anesthesia:
-
-

DEMENTIA

- Major Neuro cognitive disorder in DSM- 5
- Dementia is a disease process characterized by Progressive and generalised impairment of INTELLECTUAL

..... and

.....

EPIDEMIOLOGY-

Prevalence

- 5% >65yrs
- 20-40% > 85yrs

COMMON FEATURES OF DEMENTIA

a. Impairment of memory

Recent memory >> Remote memory

b. Impairment of language

Perseveration & Echolalia is common

c. Impairment of intellectual functions

- Apraxia →
- Agnosia →
- Prosopagnosia →
- Anosognosia →
- Aphasia →
- & Dysphasia →
- Dysarthria →
- Acalculia →

ALZHEIMER'S DISEASE

-
- Disease of the century

👉 Risk factors

- Old age
-educational status
- Head trauma
- Hypertension
- Hyper (↓ level of vitamin)
- Familial causes or Risk factors

☞ Cause of Early onset Familial variety

-
-
-

☞ Cause of late onset Familial variety

- Apo -
- Gene responsible for sporadic cases-
- UBIQUITIN-1 (ch 9)

♦ PROTECTIVE FACTORS-

EPIDEMIOLOGY

.....

.....

♦ PATHOLOGY IN ALZHEIMER'S

- Atrophy of
-
- Dilatation of

♦ NEUROPATHOLOGICAL FINDING

- → Made of protein
- Neuritic plaque

MIST

VASCULAR DEMENTIA

(Multi infarct Dementia)

- Small & Medium vessels of cerebral cortex
- Characterized by

1.onset

2. Acute

Typically this dementia progress in

Criteria for Vascular Dementia

PICK'S DISEASE OR FRONTO TEMPORAL DEMENTIA

- Age → 60yrs
- Characterized by primary progressive non and memory loss
- Along with disinhibited personality

Or

- Personality - Behaviour changes are seen

♦ NEUROPATHOLOGY

- Aggregates of cyto skeletal elements inside neurons



PICK'S BODIES

LEWY BODY DEMENTIA

- 2nd M.C type
- Type of which is a type of neuro degenerative dementia.

♦ 3 types of Synucleinopathy-(Alpha - Synucleinopathy)

- Lewy body Dementia
- Parkinson's disease
- MSA (Multisystem atrophy)

👉 FEATURES

1. Fluctuating levels of cognition → subcortical memory is affected first, later

.....

.....

2. Recurrent; Detailed well formed - persistent

(Pt..... entering in their house)

3. REM- SLEEP BEHAVIOR DISORDER

..... or in without
muscle Atonia.....acting out of dream)

4. Spontaneous motor symptoms of

..... symptoms (EPS)

- Cogwheel rigidity
- Bradykinesia
- Ataxia
- Short shuffling gait
-

Or

-

Active space

MIST

5. Severe sensitivity or
.....syndrome-

Pt. develops severe EPS even with low dose of

👉 Other features

- Recurrent fall
- Sudden unexplained loss of consciousness
- Delusion of persecution & capgras Syndrome

♦ EPIDEMIOLOGY

♦ NEUROPATHOLOGY

- Intra neuronal aggregates of
seen LEWY BODY

♦ NEURO TRANSMITTER RESPONSIBLE FOR DEMENTIA---

1.

2.

TREATMENT OF DEMENTIA

1. Central Acetyl choline esterase inhibitor

-
-
-

2. Memantine (Moderate to severe)

- antagonist

3. PDE-1 inhibitor

(PHOSPHODIESTERASE -1 INHIBITOR)-.....

ORGANIC AMNESTIC SYNDROME

- Amnestic disorder is the disorder in which is the only & pre dominant feature.

👉 Clinical features

❑ Impairment of Recent memory

- ❑ Problem in learning new things
- ❑ Problem in recalling past experiences

Immediate memory & immediate Recall / consciousness is Normal

❑ CAUSES OF AMNESIA -HMT

.....

- ❑ When amnesia occurs due to -
 Acute or chronic Thiamine deficiency

👉 Acute Thiamine deficiency

- Wernicke's Syndrome
- Acute neurological disorder
- Acute alcoholic encephalopathy

👉 Chronic Thiamine deficiency

➤ Korsakoff's syndrome

Chronic alcohol use → is the most common cause of precipitating Thiamine deficiency which cause decrease metabolism of glucose and cause Accumulation of Glutamate



Excitatory damage to neurons

❑ WERNICKE'S SYNDROME

👉 Tetrad of SYMPTOMS

- G - Global
- O- ophthalmoplegia
Bilateral
- A - Ataxia of gait
-

👉 Other symptoms

- Foot drop
- Tingling numbness
- Weakness of limb

👉 Investigations

- MRI - Brain

..... Bilateral symmetrical &
.....in the following structures.

H.

M.

T

.....is not involved in WERNICKE'S

👉 TREATMENT

- High dose of Thiamine 1000mg IV initially



100mg tablets TDS × 2 weeks

- ❑ KORSAKOFF'S SYNDROME



MEMORY

It is a reactive process involvingand
.....of information.



Workbook Questions

Q.1 Which among the following deficiency is the reason for cognitive impairment in the old age ?

- A. Vitamin C
- B. Vitamin B12
- C. Vitamin B6
- D. Vitamin B2

Q.2 Which of the following clinical features can be seen in Korsakoff's psychosis-

- 1. Ophthalmoplegia
- 2. Amnesia
- 3. Peripheral neuropathy
- 4. Confabulation

- | | |
|----------|------------|
| A] 1,2,4 | B] 1,2,3,4 |
| C] 1,3,4 | D] 2,3,4 |

Q3 All of the following statements are correct except?

- A. Opioid withdrawal is rarely fatal
- B. Buprenorphine can be used for management of opioid withdrawal
- C. Flumazenil is used for the management of long term management of Alcohol Dependence syndrome
- D. Cannabis withdrawal is associated with minimal withdrawal symptoms

Q4 A 28-year-old male is brought to the emergency department after being found unconscious at home. On examination, he is unresponsive with shallow breathing, a respiratory rate of 6 breaths per minute, pinpoint pupils, and a weak pulse. His friend reports a history of opioid use. Which of the following is the most appropriate immediate pharmacologic intervention?

- A. Flumazenil
- B. Naloxone
- C. Atropine
- D. Naltrexone

Q5 A cancer patient developed symptoms of Delirium post anesthesia. Which 1st generation antipsychotic is used in the initial management of this condition?-

- A] Lorazepam
- B] Aripiprazole
- C] Haloperidol
- D] Clozapine



First disorder is Pervasive developmental disorder (PDD)

It is type of

New name of PDD in DSM 5

AUTISM

First Pervasive developmental Disorder is Autism

It is a

It is also known as

It is more common in related to

Autism is usually diagnosed before of age and is known as infantile Autism

Diagnosis of Autism

It is diagnosed based on two symptoms

1. When there is impairment in
and
2. pattern behavior like

About 70% children of autism has

50% of children with autism develop seizure disorder by the age of 20

Child with autism has different pattern of 'finger print' k/as-

.....

MANAGEMENT----

RETT'S SYNDROME

- Is was discovered by Andreas Rett's in 1965 and only seen in girls
- It is disorder
- Gene associated with rett syndrome is
- In Retts syndrome there is normal pre-peri-postnatal development till of age
- There is normal size of head at
- After period of 5 months there is of head growth from months up to months is ultimately is seen
- There is regration of development millstones
- Loss of purposeful hand motor skills
- Repetitive stereotyped behavior
- Episode of breath holding spells which sometimes produces 'Cardiac Arrhythmia' this is the cause of death in Retts syndrome
- Abnormal EEG is seen in cases
- Seizure disorder is seen in

➔ Treatment

ASPERGER'S SYNDROME

- It is also called as high functioning Autism
- There is marked impairment in 'social interaction' because of their repetitive stereotyped behavior
- No impairment of language
- IQ is normal

Chromosome 2,7,22 is associated with Autism spectrum disorder

ADHD (ATTENTION DEFICIT HYPER ACTIVITY)

- Also k/as-.....
- ADHD is characterized by behavior'
- Multiple must start by the age of according to ICD-11 and DSM-5.
- And this hyperactive inattentive symptom should be present at least 2 areas of life..... for at

👉 Treatment

- Drug of choice is-.....
- It is a stimulant drug given after 6 years of age
- Second drug of choice is NON STIMULANT DRUG

👉 If you do not treat ADHD- child may develop
.....

DEVELOPMENTAL LEARNING DISORDER

Known asin DSM-5

Child is having significant and specific problem is learning' 'Academy skills' in reading,..... writing, and mathematical skills

Child may not.....

IQ=

CONDUCT DISORDER

- It is persistent and repetitive behavior in child of 6-12 yrs of age in which there is followed

👉 Other features:

- often with children
- from home and
- problem
- Engaged in like rape, use of weapons, gun
- towards
- Steal things do robbery
- Frequently CON others

MENTAL RETARDATION

It is known as in DSM-5

It is known as in ICD-11

Other names are Oligophrenia

There are 4 types of mental retardation

👉 IQ-

Intelligence quotient and its formula is $\text{mental age} / \text{chronological age} \times 100$

Normal IQ is between.....

Cause of mental retardation

Most common cause is

Most common genetic cause is

👉 Management

.....

.....is used to train mentally retarded person

Dysruptive Mood Dysregulation Disorder -

Workbook Questions

Q.1 A 7 year old child with history of anger episodes many times/everyday in a week, out of proportion to situation, in between episodes he has been having irritable mood for most of times for last 1 year...Likely diagnosis?

- A. DMDD (Disruptive mood dysregulation disorder)
- B. ODD (Oppositional Defiant disorder)
- C. CD (Conduct disorder)
- D. ADHD (Attention Deficit Hyperactivity Disorder)

Q.2 A 15 year boy has difficulty in expressing himself in writing and makes frequent spelling mistakes. He is not getting good grades in his exams. He is likely to be suffering from-?

- A] Mental retardation
- B] Lack of interest in studies
- C] Specific learning disorder
- D] Examination anxiety

Q.3 Which of the following is not included in diagnosing autism spectrum disorder?:

- A] Problem in socialization
- B] Abnormality in social communications
- C] Cognitive dysfunction
- D] Restricted pattern of behavior

Q.4 Which of the following SNRI used in the treatment of ADHD?

- A] A. Modafinil
- B.] Methylphenidate

- C.] Guanfacine
- D.] Reboxetine

Q.5 If child with ADHD goes untreated into adolescence, what is he/she most likely to develop?

- A] Conduct disorder
- B] Anorexia nervosa
- C] Selective mutism
- D] Autism



Active space



- M.C psychiatric disorder
- Prevalence: 17.7%

PANIC DISORDER

❑ PANIC ATTACK

- ◆ Sudden unpredictable (out of blue) attack of
often accompanied by of the following symptoms

1.
2.
3.
4.
5.
6.
7.
8.
9.
-

- ◆ If at least one panic attack is followed by one or Both of the following for at

"Panic Disorder"

- a. Excessive about next & its
- b. Significant of the patient related to ..
.....

❑ MEDICAL DISORDER WHICH PRODUCE PANIC LIKE SYMPTOMS

- Anaemia - Hb
- Hypoglycaemia - Blood sugar
- Angina - ECG
- Hyperthyroidism- T3, T4, TSH
- Pheochromocytoma
- Mitral valve prolapse
- Seizures
- Diabetes
- Alcohol with drawal

👉 TREATMENT

- Anti-depressant (SSRI) - DOC
DOC for acute Panic attack - Benzodiazepines

PHOBIA

- Fear +

❑ AGORAPHOBIA

- Agora - Market place
- Pt. is having from of the following situation from which or be available

1.
2.
3.
4.
5.
-

Active space

❑ SPECIFIC PHOBIA-----

Fear from specific, places, objects or situation.

- Acrophobia -
- Claustrophobia -
-
- Nomophobia.....
- Social Phobia.....
- Nyctophobia -
- Xenophobia -
- Cynophobia - Fear of dog

👉 Treatment

❑ Behaviour therapy

1. Systematic Desensitization- M.C. used behaviour therapy



By wolpe

- Based on principle of

Or

.....

👉 STEPS

1. Relaxation exercise
2. Make list of phobic situations
3. Desensitization

GENERALISED ANXIETY DISORDER (GAD)

- Excessive.....which is persistent and generalised about multiple

- activities of life

Like - etc.

Workbook Questions

- Q.1 Fear of seeing tall building and height is k/as?
- [A] Claustrophobia [B] Social anxiety disorder
[C] Agoraphobia [D] Acrophobia
- Q.2 Panic attack is associated with a disturbance in all of the following neurotransmitters except-
- [A] Serotonin [B] Glutamate
[C] GABA [D] Dopamine
- Q.3 Most common phobia-
- [A] Acrophobia [B] Specific phobia
[C] Photophobia [D] Agoraphobia
- Q.4 A 45 year old women presents with feeling of nervousness, abdominal pain and diarrhoea. She has history of heartburn, and indigestion for 25 years. She lives in the family where everyone remains "tense and nervous", what else symptoms do you expect to see in this condition?
- [A] Ideas of reference [B.] Hallucinations
[C.] Tingling of extremities [D.] Neologism
- Q.5 A medical student could not deliver seminar, fearing seniors. He is aware of irrationality of his fear. Likely diagnosis is-
- [A] Agoraphobia [B] Social phobia
[C] Claustrophobia [D] Specific phobia



- OBSESSION

..... = COMPULSION

Both obsession & compulsion are "Ego "

- OCD is Disorder of

- ETIOLOGY

1. Dysregulation of

2. Brain areas involved are

.....

(Head of → Striatum)

.....

.....

3. NEURO - CIRCUITORY

—



Striatum

.....



Cortex

(C-S-T-C-S)

4. Neuro-Immunological Link of OCD-

5. Psychodynamic Etiology -----

6. MRI Brain Shows

7. Most common complication of OCD ---is ---

👉 TYPES OF OCD-

1.

Leads to compulsion of

2. Pathological / Obsessional



Compulsion of

3. Pure obsession ;

Blasphemous thoughts- Sexual thoughts

👉 TREATMENT

Latest clinical practising guidelines of Indian Psychiatric Society (IPS)

➤ 1st line treatment of OCD is

Behavior therapy



1.

2. Pharmacotherapy

DOC for OCD -



Clomipramine

Lithium / valproate/ carbamazepine/ venlafaxine

MIST

- 2nd generation anti-psychotic drugs are used (Low dose) - As augmenting agent

FDA - Aripiprazole & Risperidone

New drug for OCD - Memantine

❑ TREATMENT RESISTANT OCD

— ELECTRO CONVULSIVE THERAPY



Psycho surgery

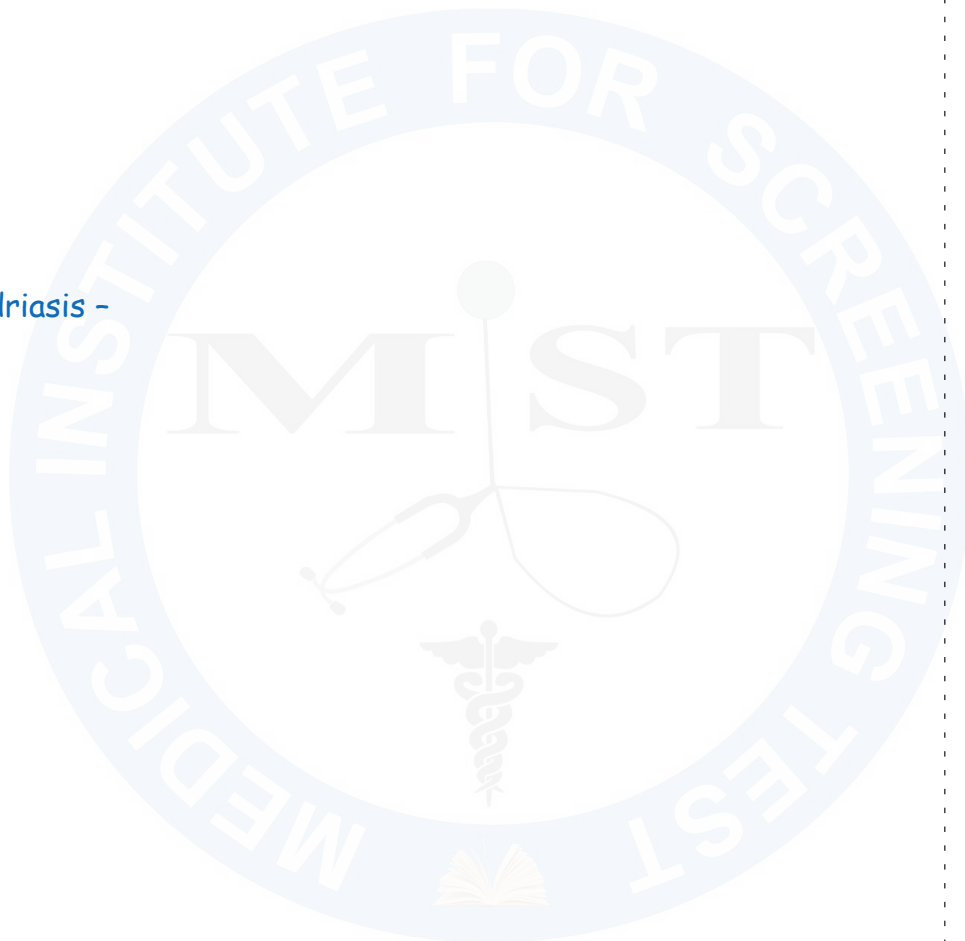


Cingulum - cingulotomy &
Sub caudate Tractotomy

BODY-FOCUSED REPETITIVE BEHAVIOR DISORDERS-

Body dysmorphic disorder -

Hypochondriasis -



Active space

Workbook Questions

Q.1 A lady washes her hands at least 50 times per day no apparent reason. Cognitive behavior therapy for her treatment should include-

- [A] Exposure
- [B] Response prevention
- [C] Thought stopping
- [D] Desensitization

Q.2 A child with OCD, MRI Brain reveals reduced volume (atrophy) of?

- [A] Globus pallidus
- [B] Cerebellum
- [C] Putamen
- [D] Caudate Nucleus

Q.3 A girl referred to the psychiatrist by the plastic surgeon. She had visited multiple plastic surgeon requesting for full facial reconstruction. She complains of dark blemishes and spots in her face, which are not actually present. She always says that she is ugly and looks constantly in the mirror and at the back of spoon and windows. Her mother assured her of no such issues, still she persists on getting the surgery. What should the psychiatrist do?

- [A] Give fitness for surgery
- [B] Start her on SSRI's
- [C] CBT
- [D] Start a typical antipsychotics

Q.4 For severe OCD psychosurgery of choice is-

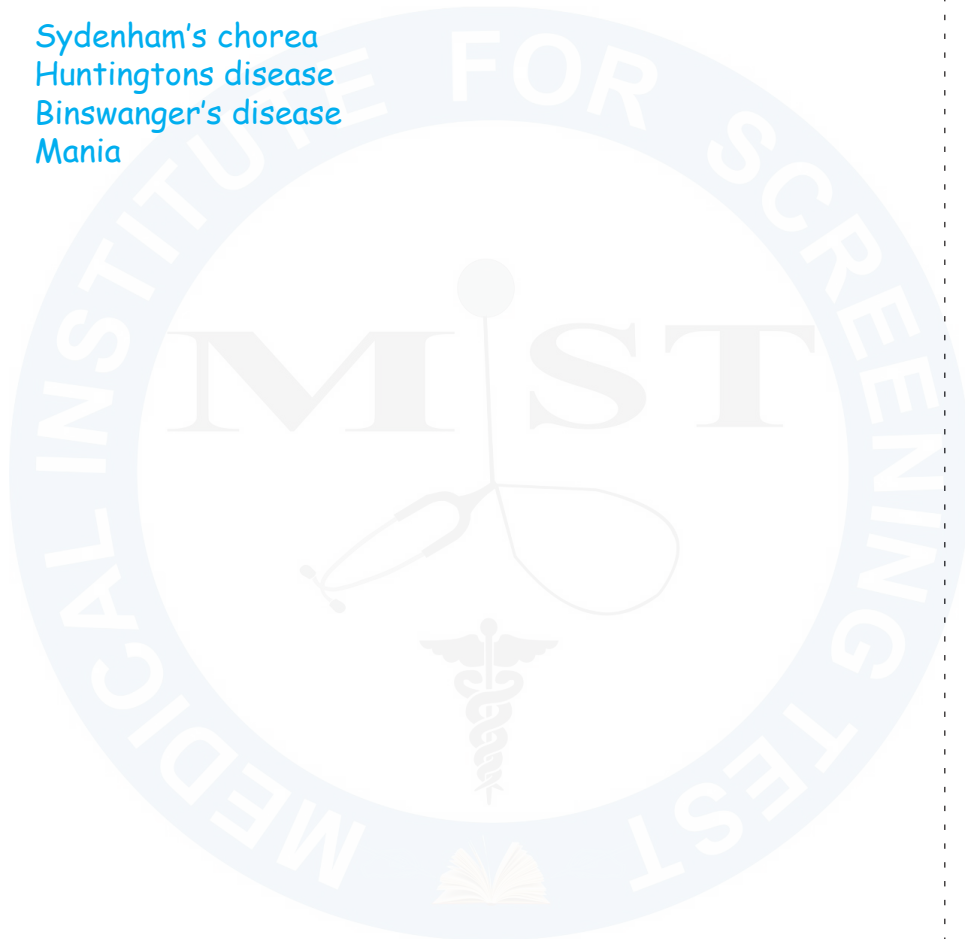
- [A] Bi-frontal tractotomy
- [B] Cingulotomy
- [C] Hypothalamotomy
- [D] None of the above

Q.5 Most common complication of obsessive compulsive neurosis is-

- [A] Mania
- [B] Depression
- [C] Dissociation of symptoms
- [D] Persecutory delusions

Q.6 Which medical condition has symptoms of OCD ?

- [A] Sydenham's chorea
- [B] Huntingtons disease
- [C] Binswanger's disease
- [D] Mania





- ♦ Most common legal/licit substance abuse in India is
- ♦ Second most common licit/legal substance abuse in India is
- ♦
- ♦ Most common illicit or illegal substance abuse in India as well as world is
- ♦ Most common psychoactive substances used worldwide is

Substance producing dependence or physical withdrawal occurs if the substance use suddenly stopped like--

Benzodiazepine

Opioids

Nicotine

Alcohol Amphetamine

Substance which produces either no dependence or very less dependence is seen with LSD

(Lysergic acid diethylamide)

According to ICD-11 Criteria for substance dependency syndrome is (>3 or more out of 6) If patient for some time during past one year Those 6 features are:

1. Craving

.....

2. Withdrawal

Mainly physiological withdrawal when use of substance Is stopped

3. Tolerance

Increased amount of substance is required to achieve previous pleasurable effect

4. Neglect of important activities for drug use

5. Persistent substance use

despite.....

because of substance use

6. Unsuccessful in or..... substance use

REVERSE TOLERANCE:

..... amount of substance is required to get previous pleasurable effect

'ALCOHOL RELATED DISORDERS'
☐ **Metabolism:**

.....of alcohol is absorbed from

90% of alcohol is metabolized in.....via oxidation

☐ **B.A.C.- Behavioral effect of alcohol:**

At particular blood alcohol concentration person shows specific behavioral changes

When concentration in 20-30mg% patient develops

.....MIST

When concentration rise to 30-80mg% patient develops relaxation,and decreased alertness

When concentration is 80-200mg%

Then they develop
motor incoordination, mood liability, slurring of speech.

When concentration reach 200-300mg% they develops nystagmus, marked slurring of speech, out

Above 300 - Coma and death

Black out- it is type of
 when person had drunk heavily and was awake,

Laboratory markers of Alcohol:

Complete blood count shows a macrocytic anemia

Liver function test shows increased ALT, increased AST, increased GGT (gamma glutamyl transferase) , increased level of CDT (carbohydrate deficient transfers)

ALT is the more specific marker for alcohol induced liver injury

GGT and CDT are the markers of

Symptoms of alcohol withdrawal:

1.

And other with this there also occurs atonomic instability, tachycardia, perspiration, anxiety, Nausea, vomiting,

2. Perceptual disturbance and psychotic symptoms:

'auditory--hallucination' and it develops
 last intake of alcohol

3. **Alcohol withdrawal Seizures** also known as Rum fits these are generalized tonic-clonic seizures it develops
 Drug of choice for alcohol withdrawal seizures is Injection-lorazepam, diazepam
Anti-epileptics especially phenytoin is not used in alcohol withdrawal seizures

4. **Delirium tremens (most severe)**

All above symptoms of withdrawal + delirium with marked Tremulousness+visual hallucinations +illusions (mistake crack in wall as snakes, floor design as insect crawling)

Delirium develops
 after last intake of alcohol

Treatment for delirium Prevention is the best treatment. Drug of choice is

Long acting benzodiazepine
 given with supplement

Management of alcohol dependent syndrome:

1.Detoxification.....

Drug of choice is

Long acting like followed by ...

.....

2. Long term maintenance



OPIOID RELATED DISORDER

- Most commonly abused opioid is also known as

♦ Signs of opioid intoxication are:

- Pinpoint pupils
- Shallow breathing or severely depressed respiration
- Bradycardia
- Hypotension
- Cold clammy cyanosed skin
- Unconsciousness
- Convulsions
- Coma and death

To reverse opioid intoxication Treatment:

intravenous

♦ Signs and symptoms of opioid withdrawal:

.....

👉 Management

1. Detoxification with the help of either

But all over the world drug of choice for Detoxification of opioid is
 Methadone

2. Long term maintenance therapy

A. Agonist therapy: also known as

Drugs:

B. Antagonist therapy: given to prevent Relapse

CANNABIS RELATED DISORDER

Also known as Marijuana

Active ingredient in Cannabis is

Various preparation of Cannabis are Ganja, charras, bhang, hashish oil (contains around 39% of THC)

We have endogenous cannabinoid in our body known as

After smoking Cannabis-Euphoric effect starts within minutes reaches to and lasts for

♦ Signs of Cannabis intoxication:

- Redness of eyes
- Tachycardia
- Dryness of mouth
- Increased appetite

.....

▪ Cannabidiol -

- 1.
- 2.
- 3.

COCAINE USE DISORDER

It is derived from the plant *Erythroxylum coca*

♦ Mechanism:

- Cocaine blocks by
- Mixture of Cocaine and heroin is known as

♦ Intoxication:

- There are Manic Like symptoms seen in Cocaine
- Elevated mood Euphoria

- Increased talkativeness
- Tachycardia.....
.....
- Mydriasis.....

Paranoid psychosis of Cocaine is seen during

Symptoms of paranoid psychosis of cocaine are:

..... and
Delusion of

HALLUCINOGEN RELATED DISORDER

Drug used:

Cocaine and Amphetamine are also kind of hallucinogen

Most potent Hallucinogen is

- ☐ the common pattern of
..... use followed by of Abstinence

TRIPS is immediate psychological effect seen after LSD use

TRIPS can be good or bad

Good trips: positive emotion, feeling happy, Euphoria, relaxation, extreme mental clarity

Bad trips: Anxiety, feeling hopelessness, suicidal ideas, panic attack, depression

INHALANT USE DISORDER

Most common Inhalant used is Toluene, petrol, whitener

TOBACCO RELATED DISORDER

Active ingredient is nicotine. Nicotine act as a stimulant

Symptoms of Nicotine withdrawal are:

.....

.....

.....

.....

Depressed mood, irritability

Craving for tobacco

Decreased concentration

Management of tobacco related disorder-

Two types of treatment available

1. Nicotine replacement therapy

Medicinal nicotine is available in the form of chewing gum, spray, patch, Lozenges

2. Non nicotine based pharmaco therapy

Two drugs are

A.

B.

..... is the marker of second hand smoke exposure

Workbook Questions

Q.1 A 20 year old girl came agitated to the emergency department with features of tachycardia, tachypnea, sweating, mydriasis and raised BP; after consuming some drug. She has previous history of suicidal tendencies. The parents do not know the mode and route of drug administration. What will be the causative drug?

- | | |
|-------------|--------------|
| [A] Morpine | [B] Cannabis |
| [C] Cocaine | [D] Heroin |

Q.2 In alcoholic man, blackouts are seen in-

- | | |
|----------------------------|----------------------------|
| [A] Alcohol Abstinence | [B] Hepatic encephalopathy |
| [C] Alcoholic intoxication | [D] Alcohol withdrawal |

Q.3 Which of the following is not true about DISULFIRAM?

- [A] Inhibits alcohol dehydrogenase
- [B] Causes accumulation of acetaldehyde
- [C] Starting dose is 250 mg
- [D] Not an anti craving (decreasing reward response to alcohol) agent

Q4 A chronic alcoholic patient, who has not consumed alcohol for last 2 days now presenting with seizures, but on investigations his liver function test were deranged. Which of the following drug would be best for him?

- | | |
|--------------|---------------|
| [A] Diazepam | [B] Midazolam |
| [C] Oxazepam | [D] Phenytoin |

Q.5 A chronic alcoholic person was drinking 750 ml alcohol for last 20 years, presented with having sensations of insects crawling on body, restlessness, insomnia and confusion. He took last drink 3 days back. What is the diagnosis?-

- | | |
|---------------------------|-----------------------|
| [A] Tolerance | [B] Reverse tolerance |
| [C] Reaction with alcohol | [D] Delirium tremens |

Q6 Tactile hallucinations seen with use of cocaine are known as:

- | | |
|-----------------|-----------------|
| [A] Formication | [B] Dipsomania |
| [C] Oneroid | [D] Carphologia |



EEG

Discovered by Hans Berger

4 TYPES OF WAVES SEEN IN EEG

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

Mnemonic-

Stages of sleep-

NREM SLEEP	REM SLEEP
1.	
2.	

◆ NREM

..... → Transition phase from wakeful to sleep

Occur only once

..... → Longest stage

Appearance of



..... → appearance of Delta waves

Stage of deep sleep

◆ REM

1st REM sleep occurs 70-90 mins..... R E M
latency time

□ NREM SLEEP DISORDERS

- Noctambulism/ somnambulism/ sleep walking-

Pt. walks during sleep with minimal motor movements

◆ NIGHT TERROR/ SLEEP TERROR(PAVOR - NOCTURNUS)

- Pt. wakes / gets up with screaming in 1st 1/3rd of sleep.

◆ BRUXISM

- Teeth grinding seen in II- NREM

◆ SOMNILOQUY (SLEEP TALKING)

- Seen in any stage of NREM sleep.

❑ DISORDER OF REM- SLEEP

- Narcolepsy
- Night mares (Dream anxiety Disorder)



Person gets up screaming after seeing of
Fearful vivid dreams in last 1/3rd of sleep ..
They remember

❑ NARCOLEPSY

- Irresistible attack of
- Pt. not able to resist sleep & there is attack of refreshing sleep for about 10-30mins.
- is the of Narcolepsy.
- 2nd M.C → CATAPLEXY
(Loss of muscle tone at heightened emotional response.)

❑ OTHER FEATURES SEEN IN NARCOLEPSY

- Hypnagogic & Hypnopompic Hallucination

◆ ETIOLOGY

- Deficiency of Hormone →



Malfunctioning of

- Genes → HLA-

HLA -



Present in

 INVESTIGATIONS

POLYSOMNOGRAPHY--Shows ↓ ed REM latency time to

- Multiple sleep Latency Test (MSLT)

 TREATMENT

DOC-

.....

Other drugs helpful in Narcolepsy are drugs which suppress REM sleep

- TCA/ Benzodiazepines



EPIDEMIOLOGY

- Prevalence- 9to15%
- Female >Male

👉 RISK FACTOR

1. Low educational status
2. Female sex
3. Past History of Psychiatric Disorder

👉 DIAGNOSIS / SYMPTOMS

If Symptoms are presents for <1month →

If > → PTSD

- Night mares
-
-
- Emotional detachment
-

- Poor concentration
- Sleep disturbance
- Irritability

👉 ETIOLOGY

- ❑ Low Volume of &

👉 TREATMENT

- Rx of choice

👉 DRUGS



ANOREXIA NERVOSA

■ EPIDEMIOLOGY

- 20:1 (F:M)
- Age of onset → 10-30yrs
- More prevalent in developed countries & seen with high frequency in engaged in profession which requires.....
..... i.e. &
- Highest mortality rate among all psychiatric disorder

■ DIAGNOSIS/SYMPTOMS

1. Significant
2. Disturbed body image-Fear of gaining weight & becoming fat.
3. weight loss achieved via
.....
.....

There is typical Pattern of handling
food..... occur.....
.....
.....

4. Loss of libido & impotency → Men
5. Amenorrhea → Females



Loss of 3 consecutive menstrual cycles

👉 OTHER FEATURES

1. Pt. is under weight
2. Lengthening of
 - ❑ QT interval - T - wave inversion

👉 TREATMENT

- ❑ Correction of Nutritional status
- ❑ NICE →
(National Institute of Clinical Excellence)

👉 "MANTRA" therapy

..... treatment in adults.

BULIMIA NERVOSA

- More prevalent than Anorexia nervosa
- Female > Male
- Weight of pt. is
- There is episode of Binge Eating



Compensatory inappropriate behaviour to stop weight gain

- Self-induced
.....
.....damage tooth enamel of the pt. → Carries/ decay
-throat & glands
- Use of laxatives
- Diuretics
- Due to Repeated Vomiting.....

👉 TREATMENT

- Correction of Nutritional status & electrolyte imbalance.
- S.....criteria used for



Active space

MIST

Workbook Questions

Q.1 Which of the following is NOT a clinical feature of (PTSD) Post traumatic stress disorder?

- [A] Flashback
- [B] Anhedonia
- [C] Hallucinations
- [D] Emotional numbing

Q.2 A mother is complaining about her young daughter that she has started behaving weird about her food habits for last few months. She eats a lot of pizzas/burgers in one go and then she vomits it out. Her BMI is normal for her age. What is the most likely diagnosis?

- [A] Binge eating disorder
- [B] Pica
- [C] Bulimia nervosa
- [D] Anorexia nervosa

Q.3 Which of the following is the most effective treatment modality for post traumatic stress disorder?

- [A] CBT
- [B] Hypnosis
- [C] Rational & emotional therapy
- [D] Eye movement desensitization & reprocessing

Q.4 Sleep spindles and K complexes are seen in NREM stage-

- [A] I
- [B] II
- [C] III
- [D] IV

Q5: An EEG recording was done in a resting normal awake patient and was asked to close eyes for some time. On waking which wave is likely to decrease?

- [A] Alpha
- [B] Beta
- [C] Delta
- [D] Theta



These disorder's are also known as
disorder also known as

1. Somatization disorder -

2. Conversion

Also known as hysteria

It is known as in
DSM-5 &
..in ICD 11

❑ Epidemiology:

10:1.F: M

MIST

👉 Other features

Sudden onset of symptoms

One or more symptoms

affecting motor or sensory function

Autonomic nervous system is typically not involved

Temporal relationship of symptoms

Primary and secondary gain

All investigation findings are in normal limit

LA-BELLE-INDIFFERENCE

Lack ofsymptom

Traumatic event which occurred in childhood is repressed into deeper layer of where it lies

..... any symptom. Another

..... later in life which is the of conversion

disorder tries to bring old repressed Traumatic material back into

consciousness; which produces anxiety and distress. This is Anxiety

gets converted into

this..... gets relieved.

Symptoms of conversion disorder

1. Sensory symptoms : blindness, deafness, midline anesthesia
2. Visceral symptoms :Diarrhea, constipation
3. Motor symptoms :weakness, paralysis, seizures,aphonic, involuntary movements, paralysis, pseudo seizures



Workbook Questions

Q.1 All are true regarding bodily distress disorder except-

- [A] Maintain sick role
- [B] 4 Pain symptoms
- [C] 1 Sexual symptoms
- [D] 1 Pseudo neurological

Q 2 A 30-year-old patient presents to the clinic 2 weeks after witnessing his father's sudden death. He reports nightmares, intrusive flashbacks, and hypervigilance. These symptoms cause significant distress but have been present for less than 1 month. Which of the following is the most appropriate diagnosis?

- [A] Post-Traumatic Stress Disorder (PTSD)
- [B] Acute Stress Disorder
- [C] Adjustment Disorder
- [D] Generalized Anxiety Disorder

Q.3 Multiple physical complaints and normal body function (According to DSM-5) is called-

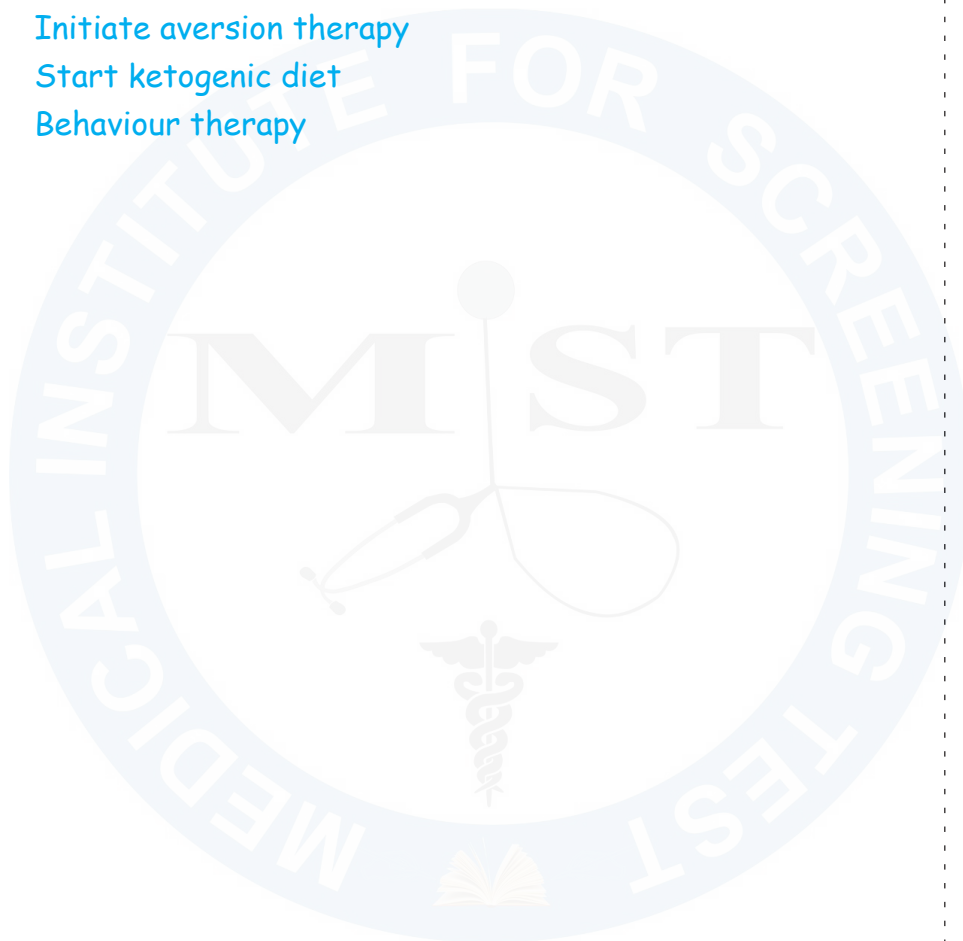
- [A] Obsession
- [B] Somatization
- [C] Somatic symptom disorder
- [D] Illness anxiety disorder

Q4 A young girl presented with sudden onset loss of vision following an accident. Ophthalmological and neurological examination was normal. On examination, she was not much concerned about blindness. Likely diagnosis?

- [A] Conversion disorder
- [B] Factitious disorder
- [C] Malingering
- [D] Somatization disorder

Q.5 A girl presented with multiple episodes of loss of consciousness. The episodes usually happen in the presence of family members in the morning. She regains consciousness after 10 to 15 minutes and usually remembers the events during the episodes. There is no history of tongue bite or urinary incontinence. What should be done in this case?

- [A] Give anti epileptics
- [B] Initiate aversion therapy
- [C] Start ketogenic diet
- [D] Behaviour therapy





SEXUAL DISORDERS

➤ Normally sexual response cycle of human has 4 stages

- D -
- E-
- O- →
- R-

Disorder of Orgasmic Phase of Sexual response cycle-

1 Delayed or absent orgasm in female Most common sexual disorder in females

2. Retarded Ejaculation-orgasm in males

3. Pre mature ejaculation →

Most disorder in Males

Cause- Psychological

Treatment..... technique

Or

SEMAN'S STOP- START TECHNIQUE

DOC - DAPOXITINE (SSRI)

ERECTILE DYSFUNCTION

- Inability to of penis which is required for intercourse in males.

- M.C cause - Psychological
- M.C organic cause - vascular
Due to Hypertension, CAD, Hypercholesterolemia

👉 TREATMENT

Most accurate treatment; Pharmacotherapy

- PDE -5 inhibitor (Phosphodiesterase 5 inhibitor)
 - Sildenafil
 - Tadalafil
 - Verdenafil
- Intracavernosal injection of Vasoactive Drugs(IIVD)
 - Papaverin
 - +
 - Prostaglandin -E1 or Alprostadil

PSYCHOLOGICAL THERAPY

- Dual therapy / Focus Technique/ sex therapy/ couple therapy
- Developed by
↓
.....
- Excessive sexual desire in females;
 - Nymphomania
- Excessive sexual desire in males
 - SATYRIASIS

PARAPHILIA'S

- Masochism -
- Eonism / transvestism -
- Necrophilia -
- Pedophilia -
- Sadism -
- Voyeurism-
- Exhibitionism ---
- Homosexuality {lesbianism}-

Workbook Questions

Q.1 During a psychoeducational evaluation, a school psychologist shows a child a picture and asks her to write a stories about them. This projective test is called ? -

Card No 4- Attitudes towards sibling rivalry



- [A] Children Apperception Test (CAT)
- [B] Rorschach Inkblot Test
- [C] Thematic Apperception Test (TAT)
- [D] Personality Inventory for Children (PIC)

Q.2 A person derives sexual pleasure with non-living things/objects, like touching shoes and undergarments. Likely diagnosis?

- [A] Voyeurism
- [B] Exhibitionism
- [C] Fetishism
- [D] Frotteurism

Q.3 TAT test measures?

- [A] Creativity
- [B] Intelligence
- [C] Neuroticism
- [D] Personality

Q.4 Premature ejaculation is disorder of which phase of sexual response cycle?

- [A] Desire
- [B] Excitement
- [C] Orgasmic
- [D] Resolution

Q.5 Clinical history of premature ejaculation in a male patient along with relationship issue with wife. Best non pharmacological management in this case will be?

- [A] Sensate focus technique
- [B] ERP
- [C] Squeeze Technique
- [D] Dapoxetine SSRI



PERSONALITY IS

.....

.....

.....

PERSONALITY DISORDERS ARE CLUBBED INTO 3 CLUSTERS

CLUSTER A - odd / aloof

CLUSTER B - Impulsive / dramatic

CLUSTER C - Anxious / fearful

CLUSTER A PERSONALITY DISORDERS

- Paranoid PD
- Schizoid PD
- Schizotypal PD

♦ PARANOID PD

-
-

♦ SCHIZOID PD

-
-
-

- Sometimes considered as a part of
-

Active space

♦ SCHIZOTYPAL

- It is classified under schizophrenia and related disorder in ICD 11
- Oddities of
- Odd
- Cognitive distortion
Odd thinking
.....
- Imaginary friends
.....
- SCHIZOTYPAL PD is common in Relatives of
-

CLUSTER B PD

1. Dis social or anti-social PD

- Frequentof society
-
-

2. Borderline PD

- In ICD 11 it is k/as-.....
- Common in
-
- short lasting
- Having multiple boyfriends or girl friends
↓
.....
- Self-harming / self-mutilatingbehaviour

- Impulsive → problem in

3. HISTRIONIC PD

- Excessive
- seeking behaviour
- Their interaction with others is often



Sexually seductive & provocative

4. NARCISSISTIC PD

- ↑
-
-
- Confused with Manic phase of Bipolar disorder

CLUSTER C PD

1. ANXIOUS - AVOIDANT P.D

-
-



Avoid

2. DEPENDANT P.D

-
-

3. ANANKASTIC P.D (OCPD)

OBSESSIVE COMPULSIVE PERSONALITY DISORDER

- No obsession +
- No compulsion

All Personality disorders are

FEATURES of ANANKASTIC PD--

- Perfectionist →
- →

👉 TREATMENT

- Rx of choice → behaviour therapy
- "DIALECTICAL BEHAVIOR THERAPY"



Borderline personality disorder

MODELS OF MIND & PSYCHO ANALYSIS

According to Dr. Sigmund Freud

TOPOGRAPHICAL MODEL OF MIND

All human experiences take place in these 3 conscious minds

1. CONSCIOUS
2. PRE CONSCIOUS
3. UNCONSCIOUS

Primary process

thinking.....

Childlike thinking which is.....

STRUCTURAL MODEL OF MIND

- Human behaviour / Functioning is guided by 3 structures of mind
 - ID
 - EGO
 - SUPER EGO
 - Defense mechanism

STAGES OF PSYCHO – SEXUAL DEVELOPMENT

PSYCHOANALYSIS

FREE ASSOCIATION:-It is uncensored of
the pt. what so ever comes without
..... their thoughts in between by the

Therapist is



Slip of

.....



(FREUDIAN)

- Pt. speaks out something which is his /her unconscious idea/ wish
- TRANSFERENCE →
- COUNTER TRANSFERENCE →

Workbook Questions

Q.1 Electra complex (given by Sigmund Freud) is seen in

- [A] Girls of 1-3 years of age
- [B] Boys of 1-3 years of age
- [C] Girls of 3-5 years of age
- [D] Boys of 3-5 years of age

Q.2 According to Sigmund Freud, primary process thinking is:

- [A] Rational
- [B] Illogical & bizarre
- [C] Logical & conscious
- [D] Seen in Pre Conscious region of mind

Q.3 A 35-year-old man is found wandering in another city by a social worker. When questioned, he appears confused about his identity and cannot recall how he arrived there. He has no signs of intoxication or head injury, and medical evaluation is normal. He is unable to provide personal information and seems to have sudden, unexpected travel away from home. What is the most likely diagnosis?

- [A] Dissociative Amnesia
- [B] Dissociative Fugue
- [C] Transient Global Amnesia
- [D] Psychogenic Non-epileptic seizures

Q.4 A patient's friend describes Mr. K as being rigid, overly focused on rules and order, and a perfectionist. He often submits work late because he insists on making it perfect and believes strongly in high moral principles. Which of the following is the most likely personality disorder?

- [A] Obsessive-Compulsive Personality Disorder (OCPD)
- [B] Narcissistic Personality Disorder
- [C] Paranoid Personality Disorder
- [D] Avoidant Personality Disorder



mhGAP—

Tele.....

NDPS ACT 1985-

❑ MENTAL HEALTH ACT →

Latest act related to mental health is

— Mental Health Care act -

Based on

— Mental hospitalnow k/as mental health

MIST

Decriminalization of suicide (IPC 309)

(Attempting suicide is not a crime; it's due to mental issue)

- DIRECT ECT →
- ECT to is prohibited, but can be given with special permission.
- "ADVANCE



1. regarding the given by pt in advance in writing.
2. (>18yr)

2 types of admission procedure

- Involuntary admission & voluntary admission both can be done up todays (.....weeks)



PROJECTIVE PERSONALITY TEST

- ❑ 1. "RORSCHACH" test



◀ FIG. 21.1 Ink Blot Test (Rorschach Test)

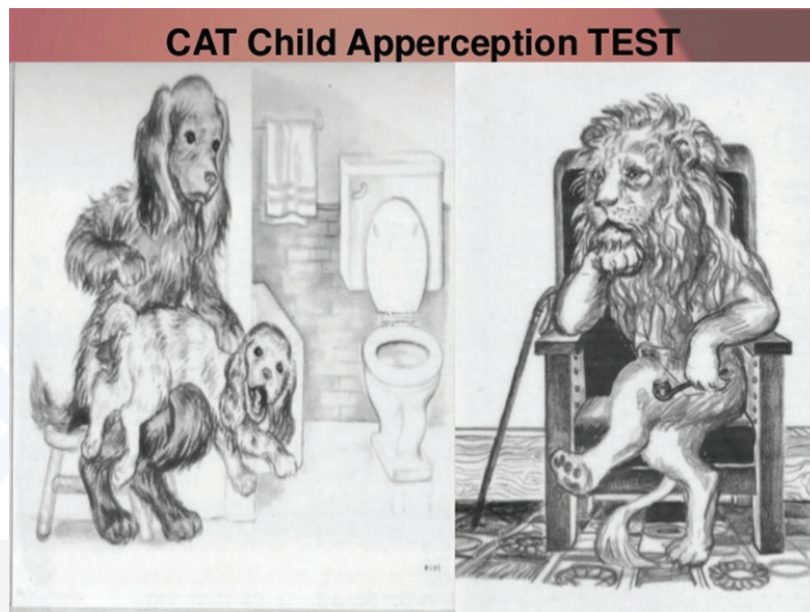
□ 2.TAT

THEMATIC APPERCEPTION TEST



Ask the pt. to write the story (2 humans-TAT)

3. CAT -CHILDREN APPERCEPTION TEST



◀ FIG. 21.3 CAT

Difference between CAT & TAT

CAT test → Animals pictures

TAT test → Human pictures

Dissociative Disorder/ Impulse Control Disorder-



Workbook Sheet



Workbook Sheet



Active space

